



Fiona Stanley Fremantle Hospitals Group

Procedure

Waitlisted Young People: Management

Reference #:FSFH-MENH-PRO-0013

Scope

Site	Service/Department/Unit	Disciplines
YouthReach South (YRS)	Cockburn Youth Centre	Medical, Nursing, Allied Health and Clerical Staff
Exclusion	Young people accepted to Ngatti House who are not engaged with a mental health service are immediately allocated a YRS clinician.	

1. Introduction

YouthReach South (YRS) is a specialist tier 4 community mental health service providing counselling, therapy and care coordination to young people aged 13 – 24 years. This document clarifies the waitlist process when a young person is accepted into YRS, but the service cannot immediately allocate a case manager.

2. Terminology

PSOLIS/webPSOLIS	Psychiatric Services Online Information System.
Mature minor	A person under the age of 18 years who has sufficient emotional and intellectual capacity to fully comprehend the nature, consequences and risks of a proposed action (e.g. treatment decision or a decision to release information) irrespective of whether a parent/substitute decision maker consents to it. A mature minor in relation to a treatment decision may either give or refuse consent.
Waitlist Manager	A clinician allocated to coordinate the waitlist monitoring process.
Ngatti House	Residential programme run by Life Without Barriers providing medium-term accommodation and clinical support to young people between 17 and 22 years of age, who show signs and symptoms of mental illness while homeless or at risk of homelessness

3. Procedure

- The Intake Meeting is held twice weekly and attended by the Triage Clinical Nurse Specialist (CNS) and the YRS multidisciplinary team (MDT).
- Referrals are presented for discussion and a decision is made whether the young person meets the eligibility criteria.
- When the YRS team is at capacity, young people accepted to the service will be waitlisted.
- The Triage CNS informs the referrer (or young person if self-referred) by phone and/or email that the young person has been accepted to YRS and is on the waitlist.
- The Triage CNS requests the referrer maintain contact with the young person while they are waitlisted and until they are activated with YRS.
- The Triage CNS reassesses waitlist prioritisation based on the acuity of the young person's mental state, severity of risk, and current biopsychosocial supports such as service providers, family, and personal support persons at each contact.
- The MDT review the waitlist at each Intake Meeting to endorse prioritisation as assessed by the triage officer.

Where the waitlisted Client has self-referred to YRS and has no community services or support person, the waitlist manager will advise the Client on appropriate pathway to acute services via CAMHS Crisis Connect for under 18yo Clients and MHERL for over 18yo

Where the Triage CNS has assessed that the client requires a timelier mental health service, they may be referred to another service such as YCATT or ATT.

- The young person will be contacted by the Triage CNS and informed that they will receive fortnightly telephone contact from a YRS mental health clinician to determine their continued willingness to engage with the service and are provided with:
 - YRS contact phone number and the name and position of the appropriate YRS contact person.
 - The 24-hour mental health crisis and emergency services numbers should there be a deterioration in mental health status or if there are any concerns about risk to self or others.

- Where the young person is not a mature minor, this information should also be provided to the legal guardian and documented in the DMR.
- The PSOLIS database will be updated to reflect the change in the client's status from "in progress" to "waitlist".
- In the event there is no direct contact/response from the young person for a period of four weeks, the YRS Triage Clinical Nurse Specialist will assess risk and inform the MDT who will plan an appropriate response.
- The referrer and the young person will be contacted to advise if the referral is closed.

3.1. Role of waitlist manager for young people with support services involved.

- The waitlist manager will make fortnightly phone contact with the waitlisted young person to:
 - remind the young person they are waitlisted to receive a service from YRS.
 - gauge young person's motivation to enter the service.
 - assess risk and reinforce the use of crisis and emergency service pathways.
- In the event of serious or imminent risk, responsible clinician should advise referrer or MH service.
- The allocated clinician is to document each contact and any additional clinical information in the DMR and PSOLIS.

3.2. Role of waitlist manager for young people without support services involved.

- The waitlist manager will make fortnightly phone contact with the young person to:
 - remind the young person they are waitlisted to receive a service from YRS.
 - gauge the young person's ongoing motivation to receive service.
 - reinforce the use of crisis and emergency pathways in the event of any serious or imminent risk.
 - provide empathic listening and supportive counselling in the context of any disclosed problems and/or distress.
 - remind the young person they can recontact the service should their mental health or biopsychosocial circumstances change.
- If it becomes evident that the young person is acutely at risk the waitlist manager will perform a Risk Assessment and Management Plan and take appropriate action in collaboration with the MDT

- The allocated clinician is to document each contact and any additional clinical information in the DMR and PSOLIS.

4. Compliance monitoring

The YRS Team Leader will monitor compliance with this procedure via clinical documentation audits at site and routine clinical incident monitoring.

5. Related Policy Documents

WA Health:

- [Statewide Standardised Clinical Documentation for Mental Health Services](#) MP 0155/21
- [Community Mental Health Status Assessments: Role of Mental Health Clinicians](#) MP 0099/18

SMHS:

- [Acute Response to mental health emergencies/crises SMHS: 17](#)

FSFHG:

- Clinical Handover FSFH-HW-POL-0035
- Clinical Documentation FSFH-HW-POL0039
- Clinical Risk Assessment and Management FSFH-HW-POL-0017

6. Related Standards

NSQHS Standards:

- Clinical Governance
- Partnering with consumers
- Communicating for Safety
- Recognising and Responding to Acute Deterioration

Chief Psychiatrists Standards for Clinical Care:

- Assessment
- Consumer and carer involvement in individual care
- Risk assessment and management

7. Authorisation

EXECUTIVE SPONSOR: Service 5 Service Director					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	07/2022	Team Leader YRS	Committee	S5 SQR Committee	12/2025
2	02/2024	Team Leader YRS	S5 SQR	S5 SQR Committee	05/2028
3 MINOR CHANGE ONLY	09/2024	Team Leader YRS	S5 SQR	S5 SQR Committee	05/2028